



Dear **Mr Sheopujan Das**,

Thank you for choosing Manipal Hospital Kharadi-pune for your diagnostic care.

Your laboratory test results are being shared following the completion of specialist review and clinical validation. Please find attached the reports that are currently authorised for release.

A brief status overview is provided below for quick reference.

As part of our commitment to keeping you informed about the status of your laboratory reports, please find the details below:

Ready – 4 | Awaited – 1 | Total – 5

Reports Summary

- Hepatitis B Surface Antigen (HbsAg) **Completed**
- HIV I II (ECLIA) **Completed**
- Kidney Profile **Completed**
- Creatinine- Serum **Completed**
- Hepatitis C Virus Antibody (HCV) **In-Progress**

You will receive an updated notification once processing is complete for the remaining reports, along with a consolidated report.

Your health information is handled with strict confidentiality and clinical oversight at every stage.

Best regards,

Manipal Hospitals Laboratory Services



Name : MR SHEOPUJAN DAS
Registration No : MH018517344
Patient Episode : O15000570562
Referred By : DR. GANESH MHETRAS
Receiving Date : 09 Jun 2026 18:31

Age : 67 Yr(s) Sex : Male
Lab No : 2603098696-1
Collection Date : 09 Jun 2026 18:29
Reporting Date : 10 Jun 2026 09:26

BIOCHEMISTRY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
CREATININE, SERUM Method: Alkaline Picrate (Jaffe)	0.82	mg/dl	[0.70-1.20] Cord : 0.6 to 1.2 Newborn(1to 4 Days)-0.3 to 1.0 Infant : 0.2 to 0.4 Child : 0.3 to 0.7 Adolescent : 0.5 to 1.0 Male : 18 -60 Yrs : 0.9 - 1.3 Female: 18 -60 Yrs : 0.6 to 1.1 Male :60 to 90 Yrs : 0.8 to 1.3 Female: 60 - 90 Yrs : 0.6 to 1.2 Male : >90 Yrs :1.0 to 1.7 Female: >90 Yrs : 0.6 to 1.3

Clinical Interpretation

Creatinine is a waste product formed in muscles from the high energy storage compound, creatine phosphate. The amount of creatinine produced is constant (unlike Urea) and is primarily a function of muscle mass. Physiological factors affecting serum creatinine concentration includes age, gender, race, muscularity, exercise, Pregnancy, certain drugs, diet, dehydration and nutritional status. Low serum Creatinine levels is seen in cases of low muscle mass like muscular atrophy, or aging. High serum creatinine levels is seen in Acute and Chronic kidney disease, obstruction. Since a rise in blood creatinine is observed only with marked damage of the nephrons, it is not suited to detect early stage

Post-dialysis sample

-----END OF REPORT-----



Dr Shoaib Khoja
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H-2015-0268
April 25, 2024-January 11, 2028
Since Jan 12-2015



Name : MR SHEOPUJAN DAS

Age : 67 Yr(s) Sex :Male

Registration No : MH018517344

Lab No : 2603095265-1

Patient Episode : O15000570562

Collection Date : 09 Jun 2026 14:32

Referred By : DR. GANESH MHETRAS

Reporting Date : 09 Jun 2026 16:48

Receiving Date : 09 Jun 2026 14:33

SEROLOGY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Hepatitis B surface Antigen (HBsAg) (ECLIA)		Specimen-Serum	
TEST RATE	0.33	COI	
CUT OFF RATE	1.0	COI	
Method:ECLIA			
			Non-reactive : Cutoff index < 0.90 Borderline : Cutoff index >= 0.90 to < 1.0 Reactive: Cutoff index >= 1.0
Test Result	Negative		

Technical Note: The hepatitis B virus is responsible for acute and chronic hepatitis infections. Acute hepatitis can be asymptomatic or present symptoms of varying severity which may progress to fulminant hepatitis. HBs antigen appears several days to several weeks after contact with the virus and can persist for several months. Persistence of HBs antigen for more than 6 months serologically defines chronic HBV infection. Disappearance of the HBs antigen is normally followed by the appearance of anti-HBs antibodies, which is a sign of recovery. Decreased HBsAg levels are found in a wide variety of conditions, including kidney disease, liver disease malabsorption, malnutrition, severe burns, infections, and cancer.

-----END OF REPORT-----

Dr R.N.RANE
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H-2015-0268
April 25, 2024-January 11, 2028
Since Jan 12-2015

Name : MR SHEOPUJAN DAS

Age : 67 Yr(s) Sex :Male

Registration No : MH018517344

Lab No : 2603095360-1

Patient Episode : O15000570562

Collection Date : 09 Jun 2026 14:32

Referred By : DR. GANESH MHETRAS

Reporting Date : 09 Jun 2026 16:41

Receiving Date : 09 Jun 2026 14:33

BIOCHEMISTRY

TEST	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
KIDNEY PROFILE			
Specimen: Serum			
UREA	152 #	mg/dl	[15-49]
Method: Urease			
CREATININE, SERUM	6.64 #	mg/dl	[0.70-1.20]
Method: Jaffe rate-IDMS Standardization			
SODIUM, SERUM	134.0 #	mmol/L	[135.0-145.0]
POTASSIUM, SERUM	4.7	mmol/L	[3.5-5.1]
CHLORIDE, SERUM	101.9	mmol/L	[98.0-107.0]
Method: ISE Indirect			
BICARBONATE, SERUM	16.1 #	mmol/L	[23.0-29.0]

Technical Note

eGFR which is primarily based on Serum Creatinine is a derivation of CKD-EPI 2009 equation normalized to 1.73 sq.m BSA and is not applicable to individuals below 18 years. eGFR tends to be less accurate when Serum Creatinine estimation is indeterminate e.g. patients at extremes of muscle mass, on unusual diets etc. and samples with severe Hemolysis / Icterus / Lipemia.

-----END OF REPORT-----

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